

ScribeAI Health

Response to MRH-RFP-2026-014 AI-Powered Ambient Clinical Documentation

Submitted to: Midwest Regional Health

Submitted by: ScribeAI Health, Inc.

Date: March 31, 2026

Account executive: Daniel Reyes, VP Enterprise

Email: daniel.reyes@scribeai.health

Direct: (415) 555-2208

1. Executive Summary

ScribeAI Health is pleased to respond to Midwest Regional Health's request for an ambient clinical documentation partner. We have spent the last seven years building the most clinician-trusted ambient documentation platform in the United States, and we partner today with thirteen of the country's twenty largest integrated delivery networks, including three with footprints comparable to MRH's.

Our platform captures the clinician-patient encounter through ambient microphone arrays, generates a specialty-aware draft note within ninety seconds of encounter conclusion, and writes the clinician-approved note back to the patient's chart through deep, bi-directional EHR integration. We support every requirement listed in Section 3 of MRH's RFP and exceed expectations across compliance, support, and clinician experience.

We have structured our response to mirror the section ordering of MRH's RFP. Pricing and contractual terms are summarized in Section 5; our standard terms appendix, included as Section 8, contains the long-form contractual language. We welcome the opportunity to discuss any item in this response with MRH's evaluation committee.

2. About ScribeAI Health

ScribeAI Health was founded in 2019 by Dr. Anya Linde, a practicing internal medicine physician, and Dr. Marcus Thorne, a machine learning researcher previously at Stanford Health AI. Our mission is to give clinicians their evenings back.

Today, our platform supports approximately 84,000 active clinicians across 190 health systems and processes more than 1.6 million ambulatory encounters each week. Our annual recurring revenue grew 168% in 2025, and we are Series D backed by Sequoia, General Catalyst, and the venture arms of three major health systems. We employ 412 people across our San Francisco headquarters and engineering hubs in Boston, Austin, and Bangalore.

3. Technical Approach

3.1 EHR integration

ScribeAI Health has a Workshop-tier partnership with Epic and an authorized integration partnership with Oracle Cerner. Our platform is available as a Hyperdrive sidebar and a Cerner Millennium embedded experience, eliminating any context switching for the clinician during the encounter.

Our Epic integration uses FHIR R4 APIs as the primary integration surface, augmented by Workshop-validated extensions for chart write-back, problem list reconciliation, and order capture. We support every Epic deployment version from 2022 forward, including Hyperdrive. We have completed live Epic deployments at 142 health systems.

Our Cerner Millennium integration uses CernerCare APIs and HL7v2 for transitional surfaces. Cerner deployment depth is, in our experience, approximately eighteen months behind our Epic integration in feature breadth, though all core write-back and identity-resolution capabilities are at parity. We have completed live Cerner deployments at 28 health systems.

3.2 Ambient capture and note generation

Encounters are captured through the clinician's existing device — phone, laptop, or wearable badge microphone — without requiring patient-side hardware. We use a proprietary speech model trained on more than 11,000 hours of medical dictation, paired with a foundation-model layer that generates the structured note. Median note latency in production is 78 seconds; 95th-percentile latency is 156 seconds.

Specialty-aware note formatting is supported across primary care, twenty-six medical and surgical specialties, and the four behavioral health subspecialty categories most commonly billed under outpatient mental health codes. We support every specialty referenced in Section 2.1 of MRH's RFP at full depth.

3.3 Languages supported

ScribeAI Health supports ambient capture in English, Spanish, Portuguese, Mandarin, and Vietnamese. We are actively piloting Hindi, Korean, and Tagalog with three of our existing customers.

3.4 Access, identity, and devices

Single sign-on is supported via Microsoft Entra ID, Okta, Ping Identity, and any SAML 2.0 or OIDC-compliant identity provider. Native iOS and Android applications are available on the App Store and Google Play. A web experience is available at provider.scribeai.health and may be embedded as a Hyperdrive sidebar.

4. Compliance and Security

ScribeAI Health operates as a HIPAA-covered Business Associate and maintains active certifications across the full set of healthcare-relevant compliance frameworks.

Framework	Status	Last verification
HIPAA Privacy and Security Rule	Compliant	Continuous; annual internal review
SOC 2 Type II	Attested by Coalfire LLC	December 2025 (report available under NDA)
HITRUST CSF r2	Certified, current	June 2025; renewal in May 2026
HITECH	Compliant	Continuous
Texas HB 300	Compliant	Continuous

4.1 Business Associate Agreement

ScribeAI Health is prepared to execute MRH's standard Business Associate Agreement as attached in Exhibit B of the RFP. Our legal team has reviewed the MRH BAA template and has identified no material concerns.

4.2 Data residency

ScribeAI Health processes and stores all customer PHI within the contiguous United States. Our primary production environment is located in the AWS us-east-1 region (Northern Virginia), with active-passive disaster recovery to AWS us-east-2 (Ohio).

We do not currently offer a customer-selectable region for primary production data. We are evaluating expansion of our footprint to additional AWS regions as part of our 2026 platform roadmap; firm timing has not yet been committed.

4.3 Encryption

All PHI is encrypted at rest using AES-256 and in transit using TLS 1.3. Encryption keys are managed through AWS KMS with customer-managed-key (CMK) options available to MRH on request at no additional cost.

4.4 Model training and customer data

ScribeAI Health does not use customer PHI or audio recordings to train, fine-tune, or otherwise improve any machine-learning artifact without the customer's explicit, prior, written, opt-in consent for each specific use. Our default position is that all customer data is processed solely for the purpose of providing the contracted Service and is not used for any other purpose.

5. Commercial Terms

5.1 Pricing

Our standard enterprise pricing for an integrated health system with MRH's footprint is as follows.

Component	Rate	Annual (3,500 providers)
Platform subscription	\$85 per provider per month	\$3,570,000
Annual support (24×7 enterprise tier)	Included	—
Implementation (one-time)	\$150,000	—
Integration build-out (Epic primary)	Included with implementation	—
Integration build-out (Cerner secondary)	\$45,000 one-time	—
Total one-time fees	—	\$195,000
Total three-year subscription value	—	\$10,710,000
Total three-year contract value	—	\$10,905,000

5.2 Contract term and renewal

- Initial contract term: three years, commencing on the date of the first production go-live
- Renewal: the contract auto-renews for successive two-year terms unless either party provides written notice of non-renewal at least 180 days before the end of the then-current term
- Year-over-year price escalation during renewal periods: 5%, compounded annually
- Termination for convenience: available to MRH after the initial three-year term with 180 days' written notice
- Termination for cause: available to either party with 60 days' written notice and a 30-day cure period

5.3 Data ownership

Clinical notes generated through the ScribeAI Health Service are the exclusive property of MRH. Audio recordings captured through the Service are the exclusive property of MRH. ScribeAI Health retains no ownership interest in either.

5.4 Data export

ScribeAI Health will provide MRH with a complete export of all clinical notes, audio recordings, and associated metadata in standard, non-proprietary formats at any time during the contract and for 180 days following termination, at no additional cost.

6. Implementation

We propose a 12-week phased implementation, beginning with a four-week pilot at two MRH locations, followed by phased expansion across the MRH footprint in eight-week waves. A dedicated Customer Success Manager and Solution Architect will be assigned to MRH for the duration of the engagement and continuing through the contract term. We will provide asynchronous and live training programs for clinicians, support staff, and IT operations teams.

7. References

ScribeAI Health serves three integrated delivery networks of comparable scale to MRH. We are pleased to make the following references available; please contact daniel.reyes@scribeai.health to arrange reference calls.

- **Cascade Health System** (Pacific Northwest, 12 hospitals, ~4,100 providers): live since Q2 2024. Reference contact provided on request
- **Atlantic Care Network** (Mid-Atlantic, 9 hospitals, ~3,200 providers): live since Q4 2024. Reference contact provided on request
- **Central Plains Health** (Midwest, 8 hospitals, ~2,800 providers): live since Q1 2025. Reference contact provided on request

8. Standard Terms (Excerpt)

8.1 Term and renewal (Section 2 of MSA)

"The initial term of this Agreement shall be thirty-six (36) months commencing on the Effective Date (the 'Initial Term'). Thereafter, this Agreement shall automatically renew for successive twenty-four (24) month renewal terms (each, a 'Renewal Term') unless either party delivers written notice of non-renewal at least one hundred eighty (180) days prior to the end of the then-current term. Subscription fees during each Renewal Term shall increase by five percent (5%) annually, compounded."

8.2 Service availability and remedies (Section 8 of MSA)

"ScribeAI Health shall use commercially reasonable efforts to maintain Service availability of 99.9% measured monthly, exclusive of scheduled maintenance windows. In the event of Service unavailability exceeding the service-level commitment in a given calendar month, Customer's sole and exclusive remedy shall be a service credit calculated as a pro-rata percentage of the monthly subscription fee, not to exceed twenty percent (20%) of the applicable monthly fee."